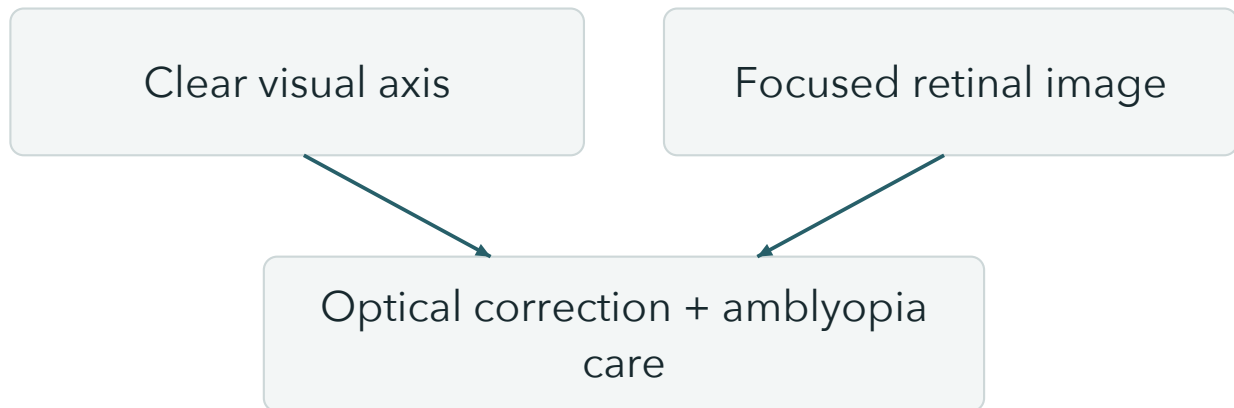


Infant cataract aphakia and lifelong risk

Part 2 | Surgery clears an axis; rehabilitation develops vision

A clear pupil alone cannot create a focused retinal image.



Removing cataract is the start of visual rehabilitation, not its endpoint.

Original visual-development chain: clear axis plus optical correction and amblyopia care.

Plan the optical correction and family support before the operation. Unilateral and bilateral aphakia present different image-size and practical problems, and amblyopia care must accompany a focused image. Document visual behaviour, refraction, alignment and adherence as the child grows. Explain that the most difficult work may continue after the wound and visual axis look satisfactory.

IATS: primary IOL versus contact-lens correction

An infant IOL did not provide the expected long-term acuity advantage.

Finding	Implication
Population	Selected infants with unilateral congenital cataract
Comparison	Primary IOL versus aphakia corrected with contact lens
Long-term acuity	No significant advantage from primary IOL
Early burden	More adverse events and additional procedures with IOL

Specify the infant population rather than extending the result to every older child or complex bilateral case. An implanted lens offers convenience but introduces different risks in a rapidly growing eye. Use the trial to discuss expected acuity and procedure burden, then assess whether the family can sustain the alternative optical correction.

Aphakic correction needs continuing adjustment

Growing eyes change the required optical power.

Finding	Implication
Contact lens	Fit, handling and wear support are important
Refraction	Reassess as axial growth changes the optics
Spectacle backup	Consider the individual unilateral or bilateral context
Adherence	A practical component of visual outcome

A successful initial fit is not a lifetime prescription. Ask how the family removes, cleans and replaces the lens, and whether interruptions leave the child without a focused image. Explain why planned refraction and fit review are necessary even when the visual axis remains clear. Link practical barriers to support rather than treating them as an unexplained poor result.

Visual-axis opacity and postoperative burden

Lens choice does not remove the need for structural review.

Finding	Implication
Axis reopacification	Can obstruct useful optical rehabilitation
Inflammatory membranes	May require additional intervention
IOL-related issues	Assess position and other complications
Blur or poor behaviour	Check optics and anatomy before assuming amblyopia alone

Separate deprivation from inadequate correction, structural obstruction and the established amblyopic component. A new reduction in visual behaviour deserves reassessment of the pathway. Discuss possible additional procedures when selecting the original operation. An explanation that attributes every disappointing result to amblyopia may miss a treatable structural problem.

Glaucoma risk remains after an IOL

Normal pressure today is not a discharge criterion.

- Maintain longitudinal pressure and structural review
↓
- Assess changes in cornea, refraction and optic nerve
↓
- Continue optical and amblyopia follow-up in parallel
↓
- Escalate suspicious findings through paediatric glaucoma care

The IATS ten-year results show accumulating glaucoma-related risk after infant surgery, without protection from primary IOL implantation. Explain why a stable secondary lens and useful vision do not cancel surveillance. Examination methods must suit the child, and pressure interpretation belongs with structural and refractive findings rather than one reassuring reading.